[redacted]

A team of experts in Diagnostics

[redacted]

Name	[redacted]	Patient ID	26320
Accession No	101	Age/Gender	75Y/F
Referred By	[redacted]	Date	06 Feb 2026

X - RAY – RIGHT KNEE AP VIEW

OBSERVATION:

- Surgical staples andICD Tube seen in situ.
- Prosthetic components of right total knee replacement are seen in situ.
- The femoral and tibial components appear well-aligned and well-seated.
- Postoperative soft tissue changes are noted around right knee joint.
- Alignment is normal.

IMPRESSION:

- Right total knee replacement with postoperative changes and prosthetic components well-positioned.
- Suggested Clinical correlation.

[redacted]

Dr ANKUR AGGARWAL
(MBBS M.D Consultant Radiologist)
MCI/09-34285

Disclaimer-It isan online interpretation of medical imaging based on clinical data. All modern machines/procedures have their own limitation. If there is any clinical discrepancy, this investigation may be repeated or reassessed by other tests. Patients identification in online reporting is not established, so in no way can this report be utilized for any medico legal purpose. In case of any discrepancy due to typing error or machinery error please get it rectified immediately

[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24810**	Registration	: 06/Feb/2026 09:00AM
UHID/MR No	: RCL0000022018	Collected	: 06/Feb/2026 09:01AM
Patient Name	: [redacted] **IPD**	Received	: 06/Feb/2026 09:01AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 06/Feb/2026 11:05AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	[redacted]	Client Code	: 315
Client Add		Barcode No	: 10083051

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
HAEMOGLOBIN (HB)				
Sample Type : Whole Blood				
HAEMOGLOBIN (HB)	10.4	g/dl	11.0-15.0	CYANMETHEMOGLOBIN

NOTE:
Hemoglobin screening helps to diagnose conditions that affect RBCs such as anemia or polycythemia.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24806**	Registration	: 06/Feb/2026 07:52AM
UHID/MR No	: RCI.0000022014	Collected	: 06/Feb/2026 07:55AM
Patient Name	: [redacted] **IPD**	Received	: 06/Feb/2026 07:56AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 06/Feb/2026 08:53AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083047

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM SODIUM				
Sample Type : Serum				
SERUM SODIUM	137.0	mmol/L	133-147	ISE

Increased In:
Dehydration, Vomiting or diarrhea, Burns or excessive sweating, Drugs: infusion hypertonic saline, Diabetes insipidus (DI), Post-traumatic, Renal failure and other renal causes.

Decreased In:
Nephrotic syndrome, cirrhosis, pancreatitis, bowel obstruction, blood loss, hypothyroidism, Adrenal insufficiency.

SERUM POTASSIUM

Sample Type : Serum

SERUM POTASSIUM	3.72	mmol/l	3.50-5.20	ISE

Increased In:
Vomiting, Diarrhea, Excessive sweating, Extensive burns, Renal failure associated with dehydration, obstruction, trauma, Renal toxicity, Addison disease, Drugs (e.g., NSAIDs, ACE inhibitors), Pseudohypoaldosteronism, use of hypertonic solutions (e.g., saline, mannitol), Intravascular hemolysis (e.g., transfusion reaction, hemolytic anemia), Rapid cellular release (e.g., crush injury, chemotherapy for leukemia or lymphoma, burns, major surgery)

Decreased In:
Uncontrolled diabetes, Renal tubular acidosis, Cushing syndrome, Malignant hypertension, Diuretics, High-dose glucocorticoids.

[redacted]

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Consultant Pathologist

[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24806**	Registration	: 06/Feb/2026 07:52AM
UHID/MR No	: RCI.0000022014	Collected	: 06/Feb/2026 07:55AM
Patient Name	: [redacted] **IPD**	Received	: 06/Feb/2026 07:56AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 06/Feb/2026 08:53AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083047

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM CREATININE				
Sample Type : Serum				
SERUM CREATININE	0.58	mg/dL	0.5-0.9	Jaffe

Increased In:

- Due to increase of creatinine crystal mass, Muscle disease: gigantism, acromegaly.
- Impaired kidney function

Decreased In:

- Pregnancy: Normal value is 0.4-0.8 mg/dL. A value >0.8 mg/dL is abnormal and should alert the clinician to further diagnostic evaluation.
- Creatinine secretion is inhibited by certain drugs (e.g., cimetidine, trimethoprim).

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24907**	Registration	: 08/Feb/2026 12:32PM
UHID/MR No	: RCI.0000022115	Collected	: 08/Feb/2026 12:33PM
Patient Name	: [redacted] **IPD**	Received	: 08/Feb/2026 12:33PM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 08/Feb/2026 03:38PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083148

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
CBC				
Sample Type : WHOLE BLOOD EDTA				
HAEMOGLOBIN (HB)	**8.3**	g/dl	11.0-15.0	CYANMETHEMOGLOBIN
TOTAL LEUCOCYTE COUNT (TLC)	6.24	10^3/uL	4.0-10.0	Flow cytometry
NEUTROPHIL	70	%	40-80	
LYMPHOCYTE	**16**	%	20-40	
MONOCYTE	08	%	2-10	
EOSINOPHIL	06	%	1-6	
BASOPHIL	00	%	<2	
ABSOLUTE NEUTROPHIL COUNT	4.36	10^3/uL	2.0-7.0	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	0.99	10^3/uL	1.0-3.0	Automated Calculated
ABSOLUTE MONOCYTE COUNT	0.49	10^3/uL	0.2-1.0	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.37	10^3/uL	0.02-0.5	Automated Calculated
ABSOLUTE BASOPHIL COUNT	**0.00**	10^3/uL	0.02-0.1	Automated Calculated
RBC COUNT(RED BLOOD CELL COUNT)	**3.17**	million/µL	3.8-4.8	Optical Flowcytometry
PCV/HAEMATOCRIT	**25.0**	%	36-46	RBC pulse height detection
MCV	**78.9**	fL	80-100	Automated/Calculated
MCH	26.1	pg	26-34	Automated/Calculated
MCHC	33.1	g/dl	32-36	Automated/Calculated
RDW-CV	12.7	%	11.6-14.0	Automated/Calculated
RDW-SD	41.9	fL	39-46	Calculated
PCT	0.225	%	0.108-0.282	
MPV	9.8	fL	7.0-12.0	Calculated
PDW	12.2	fL	9-17	Calculated
PLATELET COUNT	230	10^3/uL	150-410	Optical Flowcytometry

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

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[redacted]

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[redacted]

IP NO: 1454

Visit ID	: RCI24907	Registration	: 08/Feb/2026 12:32PM
UHID/MR No	: RCI.0000022115	Collected	: 08/Feb/2026 12:33PM
Patient Name	: [redacted] IPD	Received	: 08/Feb/2026 12:33PM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 08/Feb/2026 03:15PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083148

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST				
Sa[illegible]e Type : SERUM				
TOTAL BILIRUBIN	**1.65**	mg/dl	0.1-1.2	Diazotized, Sulfanilic
CONJUGATED (D. Bilirubin)	**0.43**	mg/dl	<0.3	Jendrassik & Grott
UNCONJUGATED (I.D. Bilirubin)	1.22			
SGOT	20	U/L	<31	Enzymatic,IFFC
S.G.P.T	14	U/L	0-34	Enzymatic,IFFC
ALKALINE PHOSPHATASE	55	U/L	30-120	Kinetic, IFCC
TOTAL PROTEINS	**6.00**	gm/dl	6.6-8.3	Biuret
ALBUMIN	**3.05**	gm/dl	3.4-4.8	BCG
GLOBULIN	2.95	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.03		1.0-2.0	Calculated
GGT	25	U/L	<38	Colorimetric Method

Note- 1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.

2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic [illegible]atitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.

3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

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[redacted]

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[redacted]

LP NO: 1454

Visit ID	: RCI24907	Registration	: 08/Feb/2026 12:32PM
UHID/MR No	: RCI.0000022115	Collected	: 08/Feb/2026 12:33PM
Patient Name	: [redacted] IPD	Received	: 08/Feb/2026 12:33PM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 08/Feb/2026 03:15PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10053148

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
EXTENDED KIDNEY FUNCTION TEST				
Sample Type : SERUM				
SERUM UREA	23.47	mg/dl	16.8-43.2	Urease GLDH
SERUM CREATININE	0.55	mg/dL	0.5-0.9	Jaffe
SERUM URIC ACID	4.64	mg/dl	2.3-6.1	URICASE
Blood Urea Nitrogen (BUN)	11.18	mg/dl	5-25	Calculated
TOTAL PROTEINS	6.00	gm/dl	6.6-8.3	Biuret
ALBUMIN	3.05	gm/dl	3.4-4.8	BCG
GLOBULIN	2.95	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.03		1.0-2.0	Calculated
SERUM SODIUM	134.9	mmol/L	133-147	ISE
SERUM POTASSIUM	3.17	mmol/l	3.50-5.20	ISE
SERUM CALCIUM	10.36	mg/dl	8.6-10.3	Arsenazo III

Urea is the end product of protein catabolism and Urea undergoes filtrations at the glomerulus as well as secretion and re absorption at the tubular level. But the level may also be affected by diet as well as certain non kidney related disorders.

Blood Urea Nitrogen can be easily calculated from the serum urea level and the serum urea levels can be easily converted to BUN by multiplying it by 0.47.Creatinine is a small tripeptide found in the muscles and is simply used as a marker of renal function. Serum creatinine is a better indicator of renal function and more specifically glomerular function than urea. Creatinine is filtered at the glomerulus and its reabsorption at the tubular level is insignificant. Because of this creatinine clearance can be used to measure Glomerular Filtration Rate (GFR).

Uric acid levels are typically 3.5-7.2 milligrams per deciliter (mg/dL) in males and 2.6-6.0 mg/dL in females. Low levels are rare, but high levels can lead to gout, kidney disease, and other health issues.

Sodium is the main determinant of **plasma osmolality**, and serum levels are closely related to hydration (volume) status. Potassium is stored almost entirely **intracellularly** and is **excreted by the kidneys**.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24907**	Registration	: 08/Feb/2026 12:32PM
UHID/MR No	: RCI.0000022115	Collected	: 08/Feb/2026 12:33PM
Patient Name	: [redacted] **IPD**	Received	: 08/Feb/2026 12:33PM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 08/Feb/2026 03:15PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted] IT	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10083148

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method

Derangements in potassium levels cause **myocardial instability**, leading to risks of potentially fatal arrhythmias.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

2026-02-04 13:03:08
Name : [redacted]
Sex :Male Age :75
Section: emergency
RoomID:
BedID:
ID:
Operator:
Custom1:
mr:
Custom3:

Data for reference only:

HR	bpm	: 94
PR Interval	ms	: 201
P Duration	ms	: 146
QRS Duration	ms	: 77
T Duration	ms	: 194
QT/QTc	ms	: 351/439
P/QRS/T Axis	deg	: 69.9/ 20.9/53.2
R(V5)/S(V1)	mV	: 0.47/0.58
R(V5)+S(V1)	mV	: 1.05

<< Conclusions >>

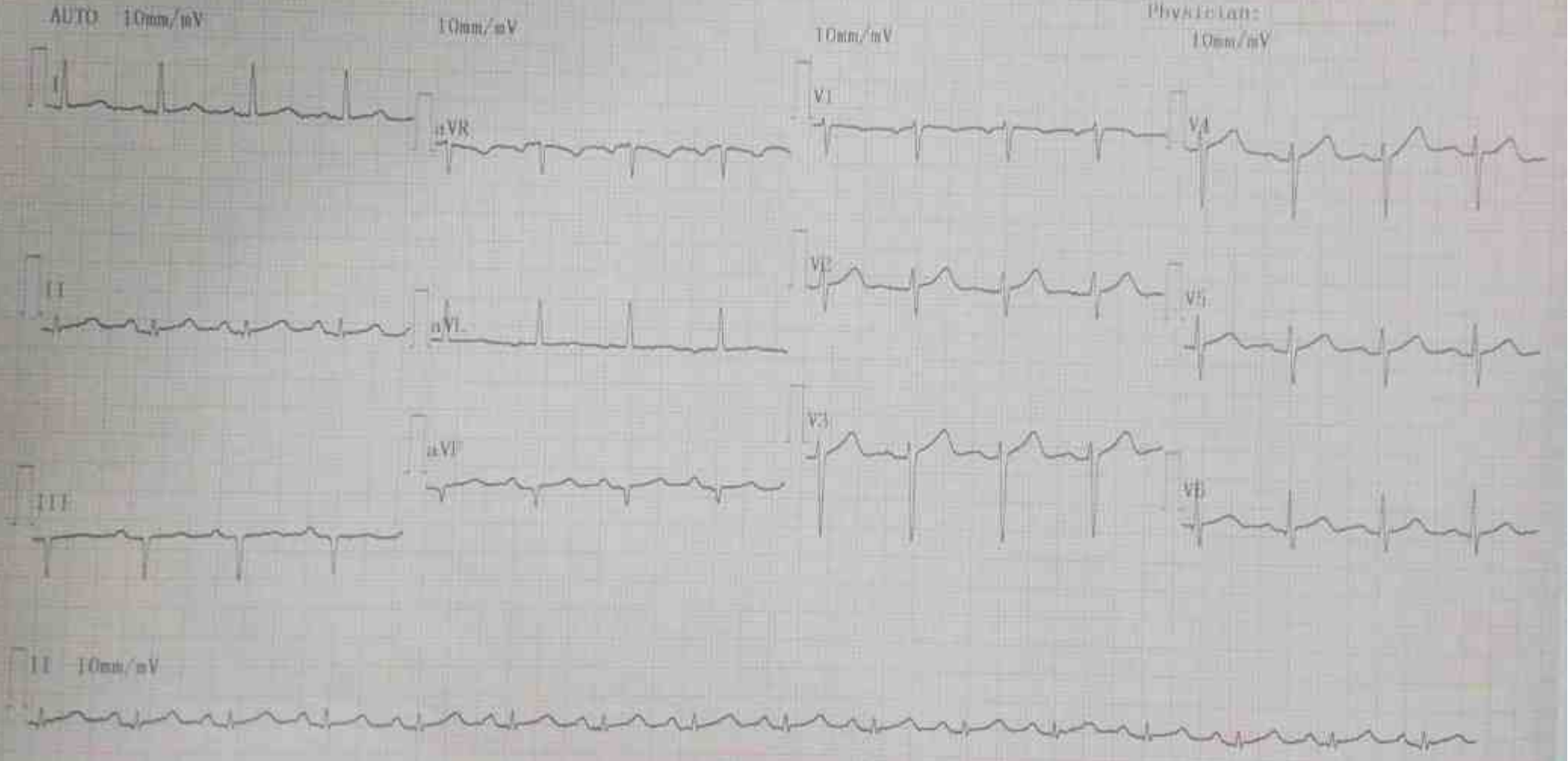
Middling left axis deviation

Report need physician confirm

IPD No. 1454
UHID 26320 (02
Name [redacted]
Age/Sex: 75 YRS / Female
DOA: 04-Feb-26
[redacted]

AUTO 10mm/mV

Physician:



25mm/sAC50Hz-EMG25Hz

[redacted]

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[redacted]

Name	[redacted]	Patient ID	25325
Age/Sex/Ht/Wt	35/F	Referred by	OPD
Exam date	[redacted]		24/12/24

X-RAY – BOTH KNEE AP, LATERAL AND SKYLINE VIEWS

OBSERVATION:

- Prominent marginal osteophytes seen around femoral condyles, tibial plateau, and patellar margins.
- Moderate obliteration of the medial and lateral joint spaces with bone-on-bone contact.
- Alignment is normal.
- No invisible abnormality is seen in the soft tissues.

IMPRESSION:

- Findings osteoarthritic changes of both knee joint (Kellgren and Lawrence grade III)
- Suggested MRI knee joint for further evaluation.

[redacted]

(Vaibhav M.D Consultant Radiologist)

MCI/69-34283

Disclaimer: It has of the interpretation of medical imaging based on clinical data. [illegible] It is a medical opinion, it is not a diagnosis. It is a report that may be repeated or reassessed by other tests. [illegible] or related to any medico-legal purpose. In case of any discrepancy [illegible]

[redacted]

[redacted]

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[redacted]

Patient	[redacted]	Age/Sex	21/M
Referred by	[redacted]	Date	15 Feb 2022

X – RAY – RIGHT LEG AP VIEW

OBSERVATION:

- Visualized parts of tibia and fibula appear normal.
- The visualized parts of ankle joint appear normal.
- Soft tissues are normal.

IMPRESSION:

Previous reports available: No

- No significant abnormality seen.

[redacted]

[redacted] M.D Consultant Radiologist]

Mr/09-34295

[redacted]

[redacted]

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[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24759**	Registration	: 04/Feb/2026 08:27PM
UHID/MR No	: RCI.0000021967	Collected	: 04/Feb/2026 08:27PM
Patient Name	: [redacted] **IPD**	Received	: 04/Feb/2026 08:27PM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 04/Feb/2026 08:31PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted] IT	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10083000

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
BLOOD GROUP ABO & RH				
Sample Type : WHOLE BLOOD EDTA				
BLOOD GROUP ABO & RH	"B" POSITIVE			

COMMENTS:

The test will detect common blood grouping system A, B, O, AB and Rhesus (RhD). Unusual blood groups or rare subtypes will not be detected by this method. Further investigation by a blood transfusion laboratory, will be necessary to identify such groups.

Disclaimer: There is no trackable record of previous ABO & RH test for this patient in this lab. Please correlate with previous blood group findings.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

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Name	[redacted]	Patient ID	[redacted]
Referred by	SSB	Age/Sex	[redacted]
Referred by	[redacted]	Date	[redacted]

X-RAY - CHEST P A VIEW

OBSERVATION:

- Prominent bronchovascular markings noted throughout the bilateral lung zones.
- The trachea is central.
- The mediastinal and cardiac silhouette are normal.
- Cardiothoracic ratio is normal.
- Cardiophrenic and costophrenic angles are normal.
- Hilums are normal.
- Bones of the thoracic cage are normal.
- Soft tissues of the chest wall are normal.

IMPRESSION:

Previous reports available: No

- Prominent bronchovascular markings noted throughout the bilateral lung zones.

Suggest clinical correlation.

[redacted]

Radiologist

MD

Disclaimer: Rule out any interpretation of medical imaging based on clinical data. [illegible] Diagnostic investigation may be repeated or reassessed by other facilities. [illegible] not intended for any medico legal purpose. In case of any discrepancy, [illegible]

[redacted]

[redacted]

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[redacted]

IP ND: 1454

Visit ID	: **RCI24746**	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCI.0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] **IPD**	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:17PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10082987

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
CP				
Sample Type : WHOLE BLOOD EDTA				
HAEMOGLOBIN (HB)	**10.5**	g/dl	11.0-15.0	CYANMETHEMOGLOBIN
TOTAL LEUCOCYTE COUNT (TLC)	6.10	10^3/uL	4.0-10.0	Flow cytometry
NEUTROPHIL	51	%	40-80	
LYMPHOCYTE	35	%	20-40	
MONOCYTE	10	%	2-10	
EOSINOPHIL	04	%	1-6	
BASOPHIL	00	%	<2	
ABSOLUTE NEUTROPHIL COUNT	3.11	10^3/uL	2.0-7.0	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	2.13	10^3/uL	1.0-3.0	Automated Calculated
ABSOLUTE MONOCYTE COUNT	0.61	10^3/uL	0.2-1.0	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.24	10^3/uL	0.02-0.5	Automated Calculated
ABSOLUTE BASOPHIL COUNT	**0.00**	10^3/uL	0.02-0.1	Automated Calculated
RBC COUNT(RED BLOOD CELL COUNT)	4.10	million/µL	3.8-4.8	Optical Flowcytometry
PCV/HAEMATOCRIT	**32.6**	%	36-46	RBC pulse height detection
MCV	**79.6**	fL	80-100	Automated/Calculated
MCH	**25.7**	pg	26-34	Automated/Calculated
MCHC	32.3	g/dl	32-36	Automated/Calculated
RDW-CV	12.9	%	11.6-14.0	Automated/Calculated
RDW-SD	42.8	fL	39-46	Calculated
PCT	0.247	%	0.108-0.282	
MPV	9.5	fL	7.0-12.0	Calculated
PDW	12.3	fL	9-17	Calculated
PLATELET COUNT	260	10^3/uL	150-410	Optical Flowcytometry

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

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[redacted]

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IP NO: 1454

Visit ID	: **RCI24746**	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCI.0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] **IPD**	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:18PM
Ref Doctor	[redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10082987

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST				
Sample Type : SERUM				
TOTAL BILIRUBIN	1.07	mg/dl	0.1-1.2	Diazotized, Sulfanilic
CONJUGATED (D. Bilirubin)	0.23	mg/dl	<0.3	Jendrassik & Grof
UNCONJUGATED (I.D. Bilirubin)	0.84			
SGOT	28	U/L	<31	Enzymatic,IFFC
S.G.P.T	22	U/L	0-34	Enzymatic,IFFC
ALKALINE PHOSPHATASE	76	U/L	30-120	Kinetic IFCC
TOTAL PROTEINS	6.89	gm/dl	6.6-8.3	Biuret
ALBUMIN	3.69	gm/dl	3.4-4.5	BCG
GLOBULIN	3.2	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.15		1.0-2.0	Calculated
GGT	26	U/L	<38	Colorimetric Method

Note- 1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.

2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.

3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

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[redacted]

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[redacted]

ISO 9001:2015 NATIONAL HEALTHCARE GLOBAL

Hospital UHID: 26/320

IP NO: 1454

Visit ID	: RCI24746	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCI.0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] IPD	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:18PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10082987

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
EXTENDED KIDNEY FUNCTION TEST				
Sample Type : SERUM				
SERUM UREA	19.05	mg/dl	16.8-43.2	Urease GLDH
SERUM CREATININE	0.59	mg/dL	0.5-0.9	Jaffe
SERUM URIC ACID	5.32	mg/dl	2.3-6.1	URICASE
Blood Urea Nitrogen (BUN)	9.07	mg/dl	5-25	Calculated
TOTAL PROTEINS	6.89	gm/dl	6.6-8.3	Biuret
ALBUMIN	3.69	gm/dl	3.4-4.8	BCG
GLOBULIN	3.2	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.15		1.0-2.0	Calculated
SERUM SODIUM	139.0	mmol/L	133-147	ISE
SERUM POTASSIUM	4.27	mmol/l	3.50-5.20	ISE
SERUM CALCIUM	10.3	mg/dl	8.6-10.3	Arsenazo III

Urea is the end product of protein catabolism and Urea undergoes filtrations at the glomerulus as well as secretion and re absorption at the tubular level. But the level may also be affected by diet as well as certain non kidney related disorders.

Blood Urea Nitrogen can be easily calculated from the serum urea level and the serum urea levels can be easily converted to BUN by multiplying it by 0.47 Creatinine is a small tripeptide found in the muscles and is simply used as a marker of renal function. Serum creatinine is a better indicator of renal function and more specifically glomerular function than urea. Creatinine is filtered at the glomerulus and its reabsorption at the tubular level is insignificant. Because of this creatinine clearance can be used to measure Glomerular Filtration Rate (GFR).

Uric acid levels are typically 3.5-7.2 milligrams per deciliter (mg/dL) in males and 2.6–6.0 mg/dL in females. Low levels are rare, but high levels can lead to gout, kidney disease, and other health issues.

Sodium is the main determinant of **plasma osmolality**, and serum levels are closely related to hydration (volume) status. Potassium is stored almost entirely **intracellularly** and is **excreted by the kidneys**. Derangements in potassium levels cause **myocardial instability**, leading to risks of potentially fatal

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[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24746**	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCI.0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] **IPD**	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:18PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10082987

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
arrhythmias				
MI				

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[redacted]

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[redacted]

IP NO: 1454

Visit ID	**: RCI24746**	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCI.0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] **IPD**	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:17PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10082987

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
PROTHROMBIN TIME				
Sample Type : SODIUM CITRATE				
PROTHROMBIN TIME				
PT Patient Value	12.9	Seconds	10-14	Clotting Assay
MNPT (Mean Normal Prothrombin Time)	13.5	Sec		
INR (International Normalized Ratio)	1.0		0.9-1.1	

INTERPRETATION:

Some of the common causes of prolonged Prothrombin time are:
Oral Anticoagulant Therapy(Therapeutic range- INR 2.5 - 3.5 recommended).
Liver Disease.
Vitamin K Deficiency.
Disseminated Intra Vascular Coagulation
Factor VII, V, X or Prothrombin deficiency.

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IP NO: 1454

Visit ID	: **RCI24746**	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCI.0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] **IPD**	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:17PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted] IT	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10082987

DEPARTMENT OF IMMUNOLOGY - SEROLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
HIV (AIDS) ANTIBODY 1 & 2 - TRIDOT				
Sample Type : SERUM				
HIV 1 & 2 - TRIDOT	NON-REACTIVE		NON-REACTIVE	

REMARKS	INTERPRETATION
Reactive	Indicates Presence of antibodies to HIV 1/2 virus
Non-Reactive	Indicates Absence of antibodies to HIV 1/2 virus

It is advised to verify all positive results by conducting the supplemental HIV 1 and HIV2 antibody confirmation and differentiation, LIA (S315).

Note 1. Positive test result indicates antibody detected against HIV-1/2.

2. Negative test result indicates antibody is not detected against HIV- 1/2.

3. Indeterminate test result indicates antibody to HIV-1/2 have been detected in the sample by two of three methods.

4. False positive results may be observed in Autoimmune diseases, Alcoholic hepatitis, Primary biliary cirrhosis,Leprosy, Multiple pregnancies, Rheumatoid factor, and due to presence of heterophile antibodies. 5. False negative results may occur during the window period and [illegible]ng the end stage of the disease.

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IP NO: 1954

Visit ID	: RCE24748	Registration	: 04/Feb/2026 01:37PM
UHID/MR No	: RCL0000021954	Collected	: 04/Feb/2026 01:37PM
Patient Name	: [redacted] IPD	Received	: 04/Feb/2026 01:37PM
Age/Gender	: 75 Y 0 M 0 D /F	Reported	: 04/Feb/2026 02:17PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10082987

DEPARTMENT OF IMMUNOLOGY - SEROLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
HEPATITIS B SURFACE ANTIGEN - RAPID CARD				
Sample Type : SERUM				
Hepatitis B Surface Antigen Rapid	NON-REACTIVE		Non-Reactive	Immunochromatography

INTERPRETATION

-Rapid (card) test for the qualitative detection of HBsAg in human serum/plasma is a sensitive and accurate one step immunoassay for the qualitative detection of Hepatitis B Surface Antigen (HBsAg). The assay is intended to be used as an aid in recognition and diagnosis of acute infections and chronic infections of Hepatitis B Virus (HBV).
-Card test can detect Hepatitis B Surface Antigen in serum or plasma at a concentration of as low as 0.5 ng/ml.
-This is only screening test. All reactive samples should be confirmed by confirmatory test. For a definitive diagnosis, the patients clinical history, symptomatology as well as serological data should be considered.
-Additional follow up testing using available clinical methods is required if card test is non-reactive with persisting clinical symptoms.

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